NUR258SCE Dysrhythmia Scenario Simulation

(Revised 12/11/2025 CA)

Client: Pamela Morales

Discipline: Adult Health Nursing II	**Student Level:** Advanced Med-Surg
Expected Simulation Run Time: 5 hours.	**Location for Reflection:** Joyce Briefing Center
Location: Joyce Simulation Center	**File Name:** NUR258SCE Dysrhythmia and Conduction Problems. NLN

Brief Description of Client

Hospital Location: Medical/Telemetry

Shift: Day shift starting at 0700 am.

Name: Pamela Morales

Date of Birth: 12/31/19XX

Age: 65

Allergies: Morphine (anaphylaxis)

Admit Weight: 91 kg

Height: 162 cm

Pronouns: She/Her

Marital Status: Married

Language: English

Sex: Female

Attending Provider/Team: Dr. Naomi Patrick

Primary Admitting Medical Diagnosis: COPD exacerbation.

Past Medical History: COPD, Hypertension, Diabetes Type 2

History of Present Illness: The client is a 65-year-old female who was admitted to the hospital two days ago for a COPD exacerbation. She has shown some clinical improvement since admission; however, her blood pressure and blood glucose levels have remained elevated. She continues to require supplemental oxygen, which she does not use at home. The client will not be eligible for discharge until she is successfully weaned off oxygen and her vital signs are stabilized.

Today marks the beginning of hospital day three.

Simulation Learning Objectives

NUR258 Course Outcomes

1. Use critical thinking, clinical reasoning, and clinical judgement to prioritize comprehensive, individualized, and holistic care of the adult client. (CO1)
2. Select appropriate priorities for nursing assessment and interventions when delivering safe nursing care for adults. (CO2)
3. Deliver evidence-based holistic nursing care for adult clients with health alterations. (CO3)
4. Evaluate outcomes of nursing care of adult clients with health alterations. (CO4)

Simulation Scenario Objectives

1. Use critical thinking to identify two markers on an EKG to determine the type of dysrhythmia the client is experiencing. (CO1)
2. Based on recognized cues, perform two appropriate interventions for the recognized cues. (CO2)
3. After taking action and performing interventions, reassess the client to ascertain if the performed interventions were successful. (CO4)

Setting/Environment

- Medical/Telemetry Unit

Equipment/Supplies for Dysrhythmia: Pamela Morales

Simulated Patient/Manikin(s) Needed: HAL

Recommended Mode for Simulator: Pre-programmed, manually advance the scenario.

Other Props & Moulage:

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Equipment Attached to Manikin/Simulated Patient:

- ID band for Pamela Morales
- Female genitalia
- IV in place and working. (saline locked)

Moulage and manikin programming:

- Oxygen in place on 2L – nasal cannula

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Equipment Available in Room:

- Equipment to take vitals: Pulse oximeter, telemetry cables (3 lead), thermometer.
- 02 delivery device: nasal cannula and simple mask.
- Toothette or toothbrush
- Alaris IV pump
- Blood collection supplies: vacutainer, gauze, tape, flushes, butterfly syringe, & lab tubes.
- Bedpan/urinal
- Black bin to deposit used supplies.
- Emesis basin
- Water cup and drinking straw.
- Suction tubing and suction canister.
- Incentive spirometer
- Grip socks

Additional Equipment Available for Scenario:

- Sequential Compression Device (SCD).

Scenario Programming

- Program cardiac rhythm to atrial fibrillation, no pauses.
- Program all lung lobes to crackles.

Vitals

HR: 142 **BP**: 99/58 **Temp**: 36.2

RR: 20 **SpO2**: 91% 2L NC

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All Medications and fluids needed for scenario (and quantity of medication)

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IV Push

- Diltiazem 0.25 mg/kg IV push, once (1 vial)
- Furosemide 40 mg IV push, BID (1 vial)

Oral Medications

- Lisinopril 20 mg, PO daily (1 tab)

Green: Need in SimServe

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IV Drip

- Diltiazem 5 mg/hr. continuous IV drip. Available in 125 mg/ 125ml from pharmacy. (1 bag)

Subcutaneous Medications

- Regular Insulin 13 units, subcutaneous, once (1 vial)

Inhalation Medications

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Yellow: in the room, running	• Albuterol Sulfate 2.5mg/Ipratropium Bromide 0.5mg SVN

Roles

Students	Simulation Staff
Lead Nurse	Nursing Instructor & SOS/Tech Team acting as following: • Physician • Charge Nurse • Pharmacy • Radiology • Cardiac/EKG Tech • Phlebotomy/Lab • Operator • Patient • Family Members

Guidelines/Information Related to Roles

All Nursing Students:

- Employee the TeamSTEPPS principles of leadership, team structure, communication, situation monitoring, and mutual respect.
- Monitor the client's condition and perform interventions as appropriate.
- Determine which assessments and interventions must be performed by the lead nurse.
- Determine and assign/delegate appropriate tasks according to the scenario.
- Set appropriate goals and objectives for scenario.
- Complete a client head to toe assessment and interpret findings of assessment, diagnostics, and lab values.
- Practice closed loop communication and SBAR.

Pre-briefing/Briefing

Students and instructor will discuss the following in pre-briefing:

1. Priorities based on client cues.
2. Possible organ dysfunction and cues.
3. Physical assessment and client cues.
4. Expected and concerning lab results.
5. Expected and concerning diagnostic results.
6. Forming a hypothesis and making recommendations based on client cues.
7. Expected medications for client.

Student Shift Report: **verbally** given to students by instructor.

Client Name	**Code Status**	**Allergies**	**DOB**	**Rm #**	**MD**
Pamela Morales	FULL	Morphine: anaphylaxis	12/31/XX **Age** 65	202	Naomi Patrick

Vital Signs (last set done at 0600 am) **HR** 118 **O2** 93% 2L NC **BP** 108/72 **Temp** 36.5 C **RR** 20	**Home Medications** • Regular insulin • Lisinopril • Hydrochlorothiazide **Health History** • Hypertension (high blood pressure) • Diabetes Type 2 • Chronic COPD

Admitting Diagnosis: COPD Exacerbation

The client is a 65-year-old female who was admitted to the hospital two days ago for a COPD exacerbation. She has shown some clinical improvement since admission; however, her blood pressure and blood glucose levels have remained elevated. She continues to require supplemental oxygen, which she does not use at home. The client will not be eligible for discharge until she is successfully weaned off oxygen and her vital signs are stabilized.
Today marks the beginning of hospital day three.

Neuro A&O x4. Stated that she was feeling a little "lightheaded." Pupils are PERRLA.	**Cardiology** Sinus tachycardia, S1&S2 BP slightly low. This is abnormal for her.
Respiratory Crackles	**GI** At 2000 last night. Continent, normal stool.
Skin/Wounds Warm, dry, and intact	**GU** 800 out since during night shift. Continent, light yellow urine.
Drains/IVs 18 gauge Left Forearm, intact.	**Diagnostics** Labs have stabilized.
Pain None	**Fall Risk** Low Fall Risk. Uses call light appropriately.
Psycho/Social Married, husband is at work.	**Diet** Cardiac healthy diet Low sodium Fluid restriction: 2L per day

ATI: INITITAL ORDERS

Medications available pre-scenario	**Medications orders**
	IV Push • Furosemide 40 mg IV push, BID (1 vial) Oral Medications • Lisinopril 20 mg, PO daily (1 tab) Subcutaneous Medications • Regular Insulin 13 units, subcutaneous, once (1 vial) Inhalation Medications • Albuterol Sulfate 2.5mg/Ipratropium Bromide 0.5mg SVN (1 nebulizer)
Orders available pre-scenario	**Nursing/Misc. orders**
	• Vital signs: every 2 hrs. • Continuous cardiac monitoring (telemetry). • Respiratory: supplemental oxygen therapy to keep SPO2 between 88-92%. • Incentive Spirometry, hourly while awake. • Sequential Compression Device (SCD): SCD on client when client is in bed.
Labs available pre-scenario	**Admission Labs**

Admission Labs

CBC	Result	Normal
WBC	15	5-10
Hgb	15	14-18
Hct	45%	42-52
Platelets	258	150-400

Chemistry/ Metabolic		
Na	146	135-145
K	3.9	3.5-5
Cl	99	98-106
CO2	25	23-29
BUN	12	10-20
Creatinine	0.9	0.6-1.2
Glucose	298	60-100
Ca	9.5	9-10.5
PO	4.0	3.5-4.5
Mg	2.1	1.3-2.3

ABG		
pH	7.30	7.35-7.45
PaO2	55	75-100
PaCO2	50	35-35
HCO3	20	22-26

Day 2 Labs

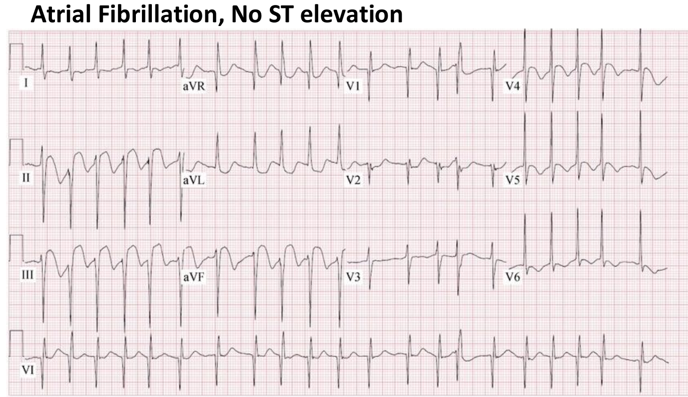
CBC	Result	Normal
WBC	11	5-10
Hgb	15	14-18
Hct	45%	42-52
Platelets	258	150-400

	<u>Chemistry/ Metabolic</u> Na 142 135-145 K 3.9 3.5-5 Cl 99 98-106 CO2 25 23-29 BUN 12 10-20 Creatinine 0.9 0.6-1.2 Glucose 298 60-100 Ca 9.5 9-10.5 PO 4.0 3.5-4.5 Mg 2.1 1.3-2.3

ATI: AVAILABLE SCENARIO ORDERS (these will all be available to push out in the scenario)

<u>Medications available to order during scenario.</u>	• 0.25 mg/kg Diltiazem IV push, once (1 dose) • 5 mg/hr Diltiazem continuous IV drip. Available in 125 mg/ 125ml from pharmacy. (1 bag)
<u>Orders available to order during scenario.</u>	• 12 Lead EKG, STAT • Troponin • CMP • ABG • Cardiology consult
<u>Lab results available to push out during scenario.</u>	**<u>Troponin Level</u>** • 0.04 (normal)

ATI: AVAILABLE SCENARIO ORDERS (these will all be available to push out in the scenario)

<u>Diagnostic Results</u>	**<u>EKG</u>** **Atrial Fibrillation, No ST elevation** 
<u>Lab results available to push out during scenario.</u>	**<u>Troponin Level</u>** • 0.04 (normal) **<u>Chemistry/ Metabolic</u>** Na 143 135-145 K 4.0 3.5-5 Cl 99 98-106 CO2 25 23-29

	BUN 13 10-20 Creatinine 1.0 0.6-1.2 Glucose 198 60-100 Ca 9.5 9-10.5 PO 4.0 3.5-4.5 Mg 2.1 1.3-2.3 **ABG** pH 7.35 7.35-7.45 PaO2 69 75-100 PaCO2 39 35-35 HCO3 22 22-26

Scenario Progression Outline

Patient Name: Pamela Morales
Doctor Name: Naomi Patrick, MD
Date of Birth: 12/31/19XX
Allergies: Morphine: anaphylaxis

Phase	Manikin/SP Actions	Expected Nursing Interventions	MD Orders and Student Cues
Phase 1	**Vitals** **HR:** 142 **BP:** 99/58 **RR:** 20 **SpO2:** 90% 2L NC **Temp:** 36.2 C **Pain:** 2/10 • Add cardiac rhythm of Afib, no pauses. • Add lung crackles in all lobes. ***NOTE: all systems are WNL, except what it listed below. • **Neuro**: client states that she feels "lightheaded" • **Cardiac**: rapid, irregular. (situation & place).	**Prior to calling the MD the students should do the following:** ❑ Review lab values. ❑ Obtain vital signs (BP, HR, O2, Temp, RR). ❑ Perform assessment. ❑ Check blood glucose: **Result 245** ❑ Form a hypothesis and plan of an SBAR with recommendations prior to calling MD. **Scenario objectives:** Use critical thinking to identify two markers on an EKG to determine the type of dysrhythmia the client is experiencing. (CO1) ❑ Irregular rhythm ❑ Loss of p-wave ❑ Hypotension ❑ Tachycardia ❑ Client states, "my heart feels fluttery." Based on recognized cues, perform two appropriate interventions for the recognized cues. (CO2) ❑ Diltiazem loading dose. ❑ Diltiazem IV drip. After taking action and performing interventions, reassess the client to ascertain if the performed interventions were successful. (CO4) ❑ Normo-tensive blood pressure. ❑ Return of NSR ❑ Client no longer has palpitations	**Student Cues** • Client complains of a flutter in her chest and feels lightheaded, like she might pass out. **MD Orders** #1 • Stat EKG, ABG, CMP. • Troponin level **MD orders after results:** • 0.25 mg/kg Diltiazem IV push, once (1 dose) • 5 mg/hr Diltiazem continuous IV drip. Available in 125 mg/ 125ml from pharmacy. • Cardiology consult

Debriefing/Guided Reflection

1. What are the next steps for the client?
2. What would you anticipate will be included in the client's discharge teaching?

Debriefing Phase	**Debriefing Questions for Consideration**
Reactions/ Defuse	How did you feel throughout the simulation experience?
	Give a brief summary of this patient and what happened in the simulation.
	What were the main problems that you identified?
Analysis/ Discovery	Discuss the knowledge guiding your thinking surrounding these main problems.
	What were the key assessments and interventions for this patient?
	Discuss how you identified these key assessments and interventions.
	Discuss the information resources you used to assess this patient. How did this guide your care planning?
	Discuss the clinical manifestations evidenced during your assessment. How would you explain these manifestations?
	Explain the nursing management considerations for this patient. Discuss the knowledge guiding your thinking.
	What information and information management tools did you use to monitor this patient's outcomes? Explain your thinking.
	How did you communicate with the patient?
	What specific issues would you want to take into consideration to provide for this patient's unique care needs?
	Discuss the safety issues you considered when implementing care for this patient.
	What measures did you implement to ensure safe patient care?
	What other members of the care team should you consider important to achieving good care outcomes?
	How would you assess the quality of care provided?
	What could you do to improve the quality of care for this patient?
Summary/ Application	If you were able to do this again, how would you handle the situation differently?
	What did you learn from this experience?
	How will you apply what you learned today to your clinical practice?
	Is there anything else you would like to discuss?

References

Hinkle, J. and Cheever, K. (2018). Brunner & Suddarth's textbook of medical-surgical nursing (14th ed.). Philadelphia, PA: Lippincott (pp.818-832)

Assessment Technologies Institute (2019). RN adult medical-surgical nursing (11th ed.). Still, KS: Author. Available from https://www.atitesting.com (pp.205-212)

Section	Content
Scenario Name	Dysrhythmia – Atrial Fibrillation with Rapid Ventricular Response (Pamela Morales)
Setting	Medical-Telemetry Unit patient admitted for COPD exacerbation
Primary Learning Focus	Recognition of atrial fibrillation with rapid ventricular response and prioritization of assessment intervention and reassessment
Priority Hypothesis	Atrial fibrillation with rapid ventricular response causing decreased cardiac output lightheadedness and hypotension in a patient with COPD exacerbation
Secondary Hypotheses	Hypoxia related to COPD fluid overload contributing to crackles metabolic stress from hyperglycemia
Key Supporting Cues	HR 142 irregular rhythm loss of P waves lightheadedness fluttering in chest BP 99/58 crackles SpO2 90 to 91 percent on oxygen
Red Flags / Escalation Triggers	HR greater than 140 irregularly irregular rhythm worsening hypotension increasing lightheadedness persistent palpitations abnormal EKG findings failure to convert after treatment
Expected Clinical Progression if Missed	Atrial fibrillation with RVR leading to worsening cardiac output hypotension decreased perfusion syncope ischemia or hemodynamic instability
Common Student Errors	Focusing only on COPD or oxygenation missing the significance of the irregular rhythm relying on monitor without getting EKG delaying SBAR or medication intervention not reassessing after diltiazem
Clinical Judgment Focus	Linking symptoms and hemodynamics to dysrhythmia identifying EKG markers of atrial fibrillation prioritizing rate control and reassessing response to treatment
Real-World Translation	This patient would require telemetry rapid provider notification 12 lead EKG rate control medication and ongoing hemodynamic monitoring with possible cardiology involvement
Instructor Teaching Emphasis	Irregular rhythm plus loss of P waves plus symptoms should trigger concern for atrial fibrillation with RVR reassessment after diltiazem is essential

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
Scenario Overview	COPD exacerbation admission	Underlying pulmonary disease affecting oxygenation and discharge readiness	Important	Provides context for hypoxia and respiratory findings	May anchor students on lungs and distract from dysrhythmia	Include COPD in overall assessment	Ongoing	B	Do not anchor on the admitting diagnosis alone		Yes
Scenario Overview	Hospital day 3	Patient condition should be improving not worsening	Important	New instability suggests new problem	Delayed recognition of change in condition	Compare current presentation to prior baseline	Ongoing	B	Clinical change on day 3 matters		No
Scenario Overview	Primary scenario focus = atrial fibrillation with RVR	Core teaching point of scenario	Immediate	This is the cause of current instability	Hemodynamic compromise syncope decreased perfusion	Recognize rhythm identify instability intervene and reassess	Ongoing	A R	This is a dysrhythmia recognition and rate control scenario	✓	Yes
Baseline / Shift Report	Full code	Background for escalation planning	Relevant	Important background only	Minimal	Know code status	N/A	S	Background awareness only		No
Baseline / Shift Report	Morphine anaphylaxis	Severe allergy with medication safety implications	Relevant	Must avoid contraindicated medication	Medication error	Verify allergies before treatment	N/A	B	Always verify allergies		No
Baseline / Shift Report	Age 65	Older adult with increased cardiovascular risk	Relevant	General context only	Minimal	Routine age-appropriate care	N/A	B	Context only		No
Baseline / Shift Report	MD Naomi Patrick	Provider for escalation and orders	Relevant	Needed for communication	Delay in escalation if unclear	Know who to call	N/A	R	Communication readiness		No
Baseline / Shift Report	HR 118 baseline	Tachycardia already present at report	Important	Shows abnormal trend before phase 1	Miss worsening to RVR	Trend HR closely	Ongoing	A	Trend from report to deterioration matters	✓	Yes
Baseline / Shift Report	O2 93 percent on 2L NC	Borderline oxygenation on supplemental oxygen	Important	Patient remains oxygen dependent	Miss hypoxia progression	Trend oxygenation	Ongoing	A	COPD patients may still deteriorate on oxygen	✓	Yes
Baseline / Shift Report	BP 108/72 baseline	Acceptable perfusion baseline	Important	Useful comparison point for later drop	Miss hypotensive trend	Trend BP	Ongoing	A	Trend awareness		No
Baseline / Shift Report	RR 20 baseline	Normal respiratory rate at report	Relevant	Useful comparison point	Miss subtle respiratory changes	Trend RR	Ongoing	A	Compare with later status		No

Dysrhythmia Pamela Morales

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	Regular insulin home medication	Background diabetes management	Relevant	Supports hyperglycemia context	Minimal	Routine unless glucose elevated	N/A	B	Home meds explain chronic conditions		No
Baseline / Shift Report	Lisinopril home medication	BP management medication	Relevant	May influence BP considerations	Potential hypotension if unstable	Monitor BP if giving	Ongoing	B	Think through BP before administering meds		No
Baseline / Shift Report	Hydrochlorothiazide home medication	Chronic BP and fluid management med	Relevant	Background only	Minimal	Routine	N/A	B	Medication relevance		No
Baseline / Shift Report	Hypertension history	Chronic cardiovascular condition	Relevant	Useful when interpreting lower-than-normal BP	Miss low BP relative to usual status	Consider baseline hypertension in assessment	Ongoing	B	Normal-looking BP may be low for this patient		Yes
Baseline / Shift Report	Diabetes type 2 history	Contributes to hyperglycemia and overall risk	Relevant	Important context	Minimal	Monitor glucose	Ongoing	B	Diabetes can worsen stress response		No
Baseline / Shift Report	Chronic COPD	Major respiratory comorbidity	Important	Affects oxygenation and respiratory assessment	Over-focusing on lungs or underestimating dysrhythmia	Assess lungs and oxygenation but maintain broad differential	Ongoing	B	Do not let COPD distract from cardiac instability	✓	Yes
Baseline / Shift Report	A&O x4 baseline	Normal baseline mentation	Important	Needed to identify later worsening or perfusion issues	Miss subtle neuro changes	Perform neuro checks	Trend LOC	A	Know baseline before changes		No
Baseline / Shift Report	Lightheadedness	Symptom of decreased cardiac output or poor perfusion	Immediate	Symptomatic dysrhythmia requires urgent evaluation	Syncope or worsening instability	Assess rhythm BP perfusion and safety	Reassess symptom after treatment	A R	Symptoms matter as much as the monitor	✓	Yes
Baseline / Shift Report	PERRLA	Normal baseline neuro finding	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low-yield baseline finding		No
Baseline / Shift Report	Sinus tachycardia from report	Previously documented rhythm before phase change	Important	Shows progression to a new rhythm later	Miss change to atrial fibrillation	Compare rhythm changes	Ongoing	A	Trend from sinus tachycardia to atrial fibrillation	✓	Yes
Baseline / Shift Report	BP slightly low for her	Perfusion may already be declining	Important	Patient-specific hypotension matters	Miss early instability	Trend BP and symptoms	Ongoing	A	Relative hypotension matters	✓	Yes
Baseline / Shift Report	Respiratory crackles	Abnormal lung sounds suggesting fluid or pulmonary congestion	Important	May worsen oxygenation and affect differential	Miss overload or respiratory contribution	Assess lungs and oxygen needs	Reassess after interventions	B	Crackles can coexist with dysrhythmia	✓	Yes

Dysrhythmia Pamela Morales

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	Last BM at 2000 normal stool	GI baseline stable	Relevant	Not central to scenario	Minimal	Routine	N/A	B	Low priority finding		No
Baseline / Shift Report	Skin warm dry intact	No acute skin issue	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority finding		No
Baseline / Shift Report	Urine output 800 overnight	Adequate output at report	Relevant	Perfusion currently adequate	Miss change if output declines later	Monitor output	Ongoing	B	Trend if instability worsens		No
Baseline / Shift Report	18 gauge left forearm IV intact	IV access available for diltiazem and labs	Important	Needed for timely intervention	Delayed treatment if not assessed	Verify IV patency	Reassess site and access	B	Always verify access before urgent meds		No
Baseline / Shift Report	Labs stabilized per report	Student may be falsely reassured	Relevant	Can distract from current deterioration	Anchoring bias	Do not rely only on prior summary; reassess current status	N/A	B	Current patient status outweighs old reassurance		Yes
Baseline / Shift Report	Pain none	No pain at baseline	Relevant	Helps distinguish palpitations/lightheadedness from pain complaint	Minimal	Routine	N/A	B	Absence of pain does not mean low risk		Yes
Baseline / Shift Report	Low fall risk uses call light appropriately	Baseline safety status	Important	Lightheadedness raises current fall risk despite prior low risk	Fall or injury	Institute fall precautions with symptom change	Reassess safety status	B	Symptoms can change fall risk	✓	Yes
Baseline / Shift Report	Married husband at work	Psychosocial context	Relevant	Background only	Minimal	Include in teaching planning	N/A	B	Holistic care		No
Baseline / Shift Report	Cardiac healthy low sodium diet	Supports chronic disease management	Relevant	Not central to acute issue	Minimal	Routine	N/A	B	Background only		No
Baseline / Shift Report	Fluid restriction 2L/day	Important context with crackles and BP meds	Important	Affects fluid decisions and pulmonary status	Overload if ignored	Factor into care planning	Reassess fluid status	B	Restrictions still matter with cardiac and pulmonary disease	✓	Yes
Pre-Scenario Orders	Furosemide 40 mg IV push available	Addresses fluid overload or congestion	Important	May help crackles but is not first treatment for atrial fibrillation	Delayed symptom improvement or over-prioritization	Use only with clinical rationale and order	Reassess lungs BP and symptoms	B	Do not treat crackles before identifying dysrhythmia		Yes
Pre-Scenario Orders	Lisinopril 20 mg PO	Antihypertensive medication	Relevant	May lower BP further if unstable	Hypotension	Consider hemodynamics before administration	Reassess BP	B	Medication judgment matters		Yes

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Pre-Scenario Orders	Regular insulin 13 units SQ	Addresses hyperglycemia	Important	Glucose contributes to metabolic stress	Delayed correction of hyperglycemia	Check glucose and administer as appropriate	Reassess blood glucose	B	Manage the whole patient but prioritize instability		No
Pre-Scenario Orders	Albuterol/Ipratropium nebulizer	Supports COPD treatment	Relevant	May be helpful for COPD symptoms but not primary issue	Anchoring on respiratory diagnosis	Treat only if clinically indicated	Reassess respiratory status	B	Not all available meds are the priority		Yes
Pre-Scenario Orders	Vitals every 2 hours	Standard monitoring order	Important	Inadequate if patient acutely unstable	Delayed recognition of worsening	Increase frequency based on patient status	Trend vitals	A	Nursing judgment can exceed routine frequency	✓	Yes
Pre-Scenario Orders	Continuous telemetry	Essential rhythm monitoring	Immediate	Core tool for detecting atrial fibrillation and response to treatment	Missed rhythm change or failed conversion	Ensure telemetry in place and interpreted	Ongoing	A	Telemetry must be interpreted not just attached	✓	Yes
Pre-Scenario Orders	Oxygen to keep SpO2 88 to 92 percent	COPD-specific oxygen target	Important	Too much or too little oxygen can be harmful	Hypoxia or inappropriate oxygen administration	Titrate oxygen to ordered range	Reassess SpO2 and work of breathing	A	COPD oxygen targets are different from standard targets	✓	Yes
Pre-Scenario Orders	Incentive spirometry hourly while awake	Supportive pulmonary order	Relevant	Not primary intervention for dysrhythmia	Distraction from urgent priorities	Use when stable	Reassess respiratory status	B	Supportive care comes after priorities		No
Pre-Scenario Orders	SCD while in bed	DVT prevention	Relevant	Standard preventive care	Minimal	Ensure use when appropriate	Ongoing	B	Basic nursing care still matters		No
Pre-Scenario Labs	Admission WBC 15	Elevated WBC at admission	Relevant	Supports inflammatory stress but not main issue today	Over-focusing on non-priority labs	Trend if needed	N/A	B	Not every abnormal lab is the priority		No
Pre-Scenario Labs	Admission Hgb 15	Normal hemoglobin	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission Hct 45	Normal hematocrit	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission platelets 258	Normal platelets	Relevant	Not central to current issue	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission Na 146	Mild hypernatremia at admission	Relevant	Abnormal but not current priority	Distraction from primary problem	Note trend	N/A	B	Abnormal does not always mean urgent		Yes

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Pre-Scenario Labs	Admission K 3.9	Normal potassium	Important	Helpful because electrolyte imbalance can cause dysrhythmia	Miss differential analysis	Review and compare	N/A	B	Normal potassium helps narrow cause		No
Pre-Scenario Labs	Admission Cl 99	Normal chloride	Relevant	Not central	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission CO2 25	Normal acid-base marker	Relevant	Background only	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission BUN 12	Normal renal marker	Relevant	Background only	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission creatinine 0.9	Normal renal function	Relevant	Important before diltiazem and meds but not urgent	Minimal	Review renal status	N/A	B	Medication safety context		No
Pre-Scenario Labs	Admission glucose 298	Significant hyperglycemia	Important	Metabolic stress may worsen condition	Delayed correction or over-focusing	Address after urgent rhythm issues	Reassess glucose	B	Important but not first priority in unstable rhythm	✓	Yes
Pre-Scenario Labs	Admission calcium 9.5	Normal calcium	Important	Normal electrolyte data support differential	Minimal	Review labs	N/A	B	Normal labs still inform judgment		No
Pre-Scenario Labs	Admission phosphorus 4.0	Normal phosphorus	Relevant	Not central	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission magnesium 2.1	Normal magnesium	Important	Normal Mg helps assess dysrhythmia causes	Minimal	Review labs	N/A	B	Electrolytes matter in dysrhythmia		No
Pre-Scenario Labs	Admission ABG pH 7.30	Acidemia at admission	Important	Supports COPD exacerbation context	Over-focusing on old data	Compare with current status	N/A	B	Use old ABG as context not final answer		No
Pre-Scenario Labs	Admission PaO2 55	Hypoxemia at admission	Important	Explains oxygen dependence	Miss chronic respiratory burden	Trend oxygenation	N/A	B	COPD context matters		No
Pre-Scenario Labs	Admission PaCO2 50	Hypercapnia at admission	Important	COPD-related CO2 retention	Inappropriate oxygen management	Use as respiratory context	N/A	B	Chronic lung disease changes targets		No
Pre-Scenario Labs	Admission HCO3 20	Low bicarbonate at admission	Relevant	Context for acid-base status	Minimal	Compare trends	N/A	B	Trend-based interpretation		No
Pre-Scenario Labs	Day 2 WBC 11	Improved from admission	Relevant	Not current priority	Minimal	Trend if needed	N/A	B	Trend awareness		No

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Pre-Scenario Labs	Day 2 Na 142	Near normal sodium	Relevant	Not current priority	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Day 2 glucose 298	Persistent hyperglycemia	Important	Needs treatment but not before unstable rhythm	Delayed glucose management	Address after urgent issues	Reassess glucose	B	Multiple problems require prioritization	✓	Yes
Pre-Scenario Orders to Release	Stat 12 lead EKG	Definitive rhythm identification	Immediate	Needed to confirm atrial fibrillation with no ST elevation	Delayed diagnosis and treatment	Obtain EKG promptly	Interpret results	A R	Do not rely on monitor alone	✓	Yes
Pre-Scenario Orders to Release	Troponin	Rules out ischemic component	Important	Part of dysrhythmia workup	Miss concurrent ACS	Send and review	Reassess with result	B	Work up the whole picture		No
Pre-Scenario Orders to Release	CMP	Evaluates metabolic contributors	Important	Helps assess electrolytes and renal status	Miss reversible contributors	Send and review	Reassess with result	B	Dysrhythmias require lab review		No
Pre-Scenario Orders to Release	ABG	Evaluates oxygenation and ventilation	Important	COPD and dysrhythmia both affect perfusion/oxygenation	Miss worsening gas exchange	Obtain ABG	Reassess with result	B	Respiratory and cardiac issues overlap		No
Pre-Scenario Orders to Release	Cardiology consult	Appropriate specialty escalation	Important	Needed for ongoing management	Delayed specialty care	Request consult	N/A	R	Know when to escalate beyond bedside care		No
Released Results	EKG = atrial fibrillation no ST elevation	Confirms rhythm diagnosis	Immediate	Determines correct intervention path	Wrong treatment or delay	Interpret EKG and update SBAR	Reassess after meds	A R	Loss of P waves plus irregular rhythm identifies AFib	✓	Yes
Released Results	Troponin 0.04 normal	No evidence of myocardial injury currently	Important	Helps narrow differential	Over-treating as MI	Continue dysrhythmia-focused plan	N/A	B	Normal troponin does not remove urgency of unstable rhythm		No
Released Results	CMP Na 143	Normal sodium on current workup	Relevant	Not priority	Minimal	Routine	N/A	B	Low priority current lab		No
Released Results	CMP K 4.0	Normal potassium on current workup	Important	Helps rule out electrolyte trigger	Miss differential reasoning	Document and proceed	N/A	B	Normal potassium helps narrow cause		No

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Released Results	CMP glucose 198	Still elevated but improved	Important	Requires management but not before rhythm stabilization	Delayed glucose care	Address after urgent rhythm issue	Reassess glucose	B	Prioritize instability first		No
Released Results	ABG pH 7.35	Normalized acid-base status	Important	Shows respiratory status improved from admission	May falsely reassure	Interpret in context of current symptoms	N/A	B	Better ABG does not remove cardiac urgency		Yes
Released Results	ABG PaO2 69	Still low oxygenation	Important	Supports ongoing oxygen need	Miss persistent hypoxemia	Continue oxygen monitoring	Reassess SpO2 and symptoms	B	COPD still matters in the background		No
Phase 1 Deterioration	HR 142	Rapid ventricular response	Immediate	Rate causing symptoms and poor filling	Worsening decreased cardiac output	Recognize instability and escalate	Trend HR	A R	Rate matters in symptomatic AFib	✓	Yes
Phase 1 Deterioration	BP 99/58	Hypotension	Immediate	Reduced perfusion from dysrhythmia	Collapse syncope decreased organ perfusion	Monitor and support BP	Reassess after treatment	A R	Hypotension makes this more urgent	✓	Yes
Phase 1 Deterioration	RR 20	Respiratory rate not extreme despite symptoms	Relevant	Do not let normal RR falsely reassure	Miss cardiac cause because breathing looks acceptable	Continue full assessment	Trend RR	B	Normal RR does not rule out instability		Yes
Phase 1 Deterioration	SpO2 90 percent on 2L	Low oxygenation on supplemental oxygen	Important	Patient remains vulnerable due to COPD	Hypoxia	Adjust oxygen within ordered range	Reassess SpO2	B	Oxygenation still matters though not primary problem	✓	Yes
Phase 1 Deterioration	Temp 36.2	No fever concern	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority finding		No
Phase 1 Deterioration	Pain 2 out of 10	Minimal pain	Relevant	Helps focus away from pain and toward perfusion symptoms	Minimal	Routine	N/A	B	Absence of pain does not equal stability		Yes
Phase 1 Deterioration	Atrial fibrillation no pauses	Rhythm pathology causing symptoms	Immediate	Direct cause of instability	Decreased cardiac output	Initiate ordered treatment and monitoring	Reassess rhythm	A R	Students must recognize AFib markers	✓	Yes
Phase 1 Deterioration	Crackles in all lobes	Abnormal pulmonary finding	Important	May contribute to dyspnea and reflect fluid status	Worsening oxygenation	Assess lungs and fluid status	Reassess lungs after interventions	B	Cardiac and pulmonary findings may coexist	✓	Yes

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Phase 1 Deterioration	Lightheadedness symptomatic	Indicates decreased perfusion	Immediate	Symptoms make rhythm clinically significant	Syncope or fall	Institute safety precautions and treat cause	Reassess symptom	A R	Symptomatic patients are not just monitor problems	✓	Yes
Phase 1 Deterioration	Rapid irregular cardiac rhythm	Classic bedside finding of AFib with RVR	Immediate	Correlates with EKG and symptoms	Delayed treatment	Perform focused cardiac assessment	Reassess after medication	A	Palpation and auscultation still matter	✓	Yes
Phase 1 Trigger Event	Client says heart feels fluttery	Patient-reported cue of dysrhythmia	Immediate	Subjective symptoms support urgent concern	Dismissed symptoms leading to delay	Take symptom seriously and correlate to rhythm	Reassess after treatment	A	Patient statements are important cues	✓	Yes
Phase 1 Expected Actions	Review lab values	Core preparation step	Important	Needed to rule out contributors and form plan	Weak hypothesis or poor SBAR	Review chart before escalation	N/A	R	Preparation matters		No
Phase 1 Expected Actions	Obtain full vital signs	Required nursing assessment	Immediate	Confirms hemodynamic status	Miss instability	Obtain and trend full vitals	Repeat after interventions	A	Assessment before escalation	✓	Yes
Phase 1 Expected Actions	Perform assessment	Full bedside assessment required	Immediate	Identifies symptom burden and instability	Delay in treatment	Assess neuro cardiac respiratory and safety	Repeat after treatment	A	Assessment drives intervention	✓	Yes
Phase 1 Expected Actions	Check blood glucose result 245	Supports diabetic management and stress response	Important	Important but not first priority over unstable rhythm	Delayed glucose management	Check and treat as ordered after urgent issues	Reassess glucose	B	Prioritization means not everything is first		Yes
Phase 1 Expected Actions	Form hypothesis and SBAR before calling MD	Organized clinical reasoning and communication	Important	Improves escalation and recommendations	Poor communication delay	Prepare concise SBAR	N/A	S A R	SBAR should include rhythm symptoms BP and recommendation	✓	Yes
Phase 1 Orders	MD orders stat EKG ABG CMP troponin	Appropriate workup for symptomatic dysrhythmia	Immediate	Needed to clarify cause and severity	Delay in diagnosis	Carry out promptly	Review results	A R	Workup is part of the intervention plan	✓	Yes
Phase 1 Orders	MD orders diltiazem loading dose	Rate control medication	Immediate	Primary treatment for AFib with RVR here	Persistent tachycardia and low output	Administer safely via IV	Reassess HR BP symptoms	A R	Know why diltiazem is used	✓	Yes

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Phase 1 Orders	MD orders diltiazem drip	Maintains ongoing rate control	Immediate	Prevents recurrence or persistent RVR	Ongoing instability	Start infusion correctly	Reassess telemetry HR BP	A R	Loading dose and drip serve different purposes	✓	Yes
Phase 1 Orders	MD orders cardiology consult	Escalation to specialty care	Important	Appropriate for ongoing rhythm management	Delay in expert management	Initiate consult	N/A	R	Know when further expertise is needed		No
Post-Intervention Reassessment	Normotensive blood pressure expected	Treatment success marker	Immediate	Shows improved perfusion	Miss failed therapy	Reassess BP frequently	Ongoing	A R	Students must know what improvement looks like	✓	Yes
Post-Intervention Reassessment	Return of NSR expected	Treatment success marker	Immediate	Indicates rhythm control	Miss persistent AFib	Reassess telemetry and rhythm strip	Ongoing	A R	Reassessment includes rhythm not just symptoms	✓	Yes
Post-Intervention Reassessment	Client no longer has palpitations expected	Symptom improvement marker	Important	Patient report confirms response	Miss persistent symptom burden	Ask directly about symptoms	Reassess after treatment	A R	Subjective improvement matters		Yes
Scenario-Wide Teaching Point	Identify two EKG markers of AFib	Core learning objective	Immediate	Needed for diagnosis	Wrong diagnosis and delayed treatment	Teach irregular rhythm and absent P waves	N/A	A	Monitor strips must be interpreted clinically	✓	Yes
Scenario-Wide Teaching Point	Reassess after diltiazem	Core medication safety principle	Immediate	Diltiazem affects rate and BP	Miss hypotension or failed conversion	Reassess HR rhythm BP and symptoms	Ongoing	A R	Every antiarrhythmic/rate-control med requires reassessment	✓	Yes
Scenario-Wide Teaching Point	Do not anchor on COPD	Common reasoning error	Important	Respiratory disease can distract from primary cardiac issue	Wrong prioritization	Maintain broad differential	N/A	B	Admitting diagnosis is not always the current problem	✓	Yes
Scenario-Wide Teaching Point	Symptomatic AFib is a perfusion problem	Key clinical judgment concept	Immediate	Links rhythm to patient symptoms	Delayed escalation	Treat rhythm and perfusion together	N/A	A	The monitor matters because the patient is symptomatic	✓	Yes

Section	Debrief Question	Instructor Prompt
Reaction / Defusing	How did you feel throughout the simulation experience?	Encourage students to identify initial emotional response before moving into analysis.
Reaction / Defusing	Give a brief summary of this patient and what happened in the simulation.	Guide students to summarize the transition from COPD context to symptomatic atrial fibrillation.
Reaction / Defusing	What were the main problems that you identified?	Prompt students to identify dysrhythmia decreased cardiac output and symptom burden as the priority problems.
Analysis / Discovery	Discuss the knowledge guiding your thinking surrounding these main problems.	Encourage connection between assessment findings EKG changes and perfusion.
Analysis / Discovery	What were the key assessments and interventions for this patient?	Guide students toward rhythm assessment vital signs EKG diltiazem and reassessment.
Analysis / Discovery	Discuss how you identified these key assessments and interventions.	Prompt students to explain which findings made the dysrhythmia clinically significant.
Analysis / Discovery	Discuss the information resources you used to assess this patient. How did this guide your care planning?	Encourage students to include report telemetry EKG labs blood glucose and bedside assessment.
Analysis / Discovery	Discuss the clinical manifestations evidenced during your assessment. How would you explain these manifestations?	Guide students toward lightheadedness fluttering hypotension and rapid irregular rhythm as signs of decreased cardiac output.
Analysis / Discovery	Explain the nursing management considerations for this patient. Discuss the knowledge guiding your thinking.	Prompt students to prioritize hemodynamic stability rhythm control oxygenation and safe medication administration.
Analysis / Discovery	What information and information management tools did you use to monitor this patient's outcomes? Explain your thinking.	Encourage students to identify telemetry 12 lead EKG repeated vital signs symptom report and lab review.

Section	Debrief Question	Instructor Prompt
Analysis / Discovery	How did you communicate with the patient?	Guide students to reflect on clear calm communication during a potentially frightening rhythm change.
Analysis / Discovery	What specific issues would you want to take into consideration to provide for this patient's unique care needs?	Prompt students to consider COPD oxygen targets hypotension diabetes and fall risk from lightheadedness.
Analysis / Discovery	Discuss the safety issues you considered when implementing care for this patient.	Encourage students to focus on hypotension fall risk medication effects and monitoring during IV diltiazem.
Analysis / Discovery	What measures did you implement to ensure safe patient care?	Guide students toward close monitoring IV patency telemetry medication checks and reassessment.
Analysis / Discovery	What other members of the care team should you consider important to achieving good care outcomes?	Prompt students to include provider cardiology pharmacy and respiratory support as appropriate.
Analysis / Discovery	How would you assess the quality of care provided?	Encourage evaluation of timeliness prioritization communication and reassessment.
Analysis / Discovery	What could you do to improve the quality of care for this patient?	Prompt students to focus on earlier recognition stronger EKG interpretation and more concise SBAR communication.
Summary / Application	What are the next steps for the client?	Guide students to consider continued telemetry cardiology follow-up rate control and discharge planning.
Summary / Application	What would you anticipate will be included in the client's discharge teaching?	Encourage students to think about medications symptom reporting follow-up oxygen use and when to seek help.
Summary / Application	If you were able to do this again how would you handle the situation differently?	Encourage students to identify one or two specific behavior changes.
Summary / Application	What did you learn from this experience?	Guide students toward a clear clinical judgment or dysrhythmia recognition takeaway.

Section	Debrief Question	Instructor Prompt
Summary / Application	How will you apply what you learned today to your clinical practice?	Prompt students to connect rhythm recognition and reassessment to future bedside care.
Summary / Application	Is there anything else you would like to discuss?	Use this as an invitation for unresolved questions or reflection.

Dysrhythmia (Atrial Fibrillation)

The scenario objectives focus on recognizing atrial fibrillation, identifying ECG markers, performing appropriate interventions, and reassessing after treatment.

Critical Cues

- ☐ Irregular rhythm
- ☐ No P waves
- ☐ Hypotension
- ☐ Tachycardia
- ☐ Patient reports “fluttering”

Expected Priorities

- ☐ Rhythm recognition
- ☐ Hemodynamic assessment
- ☐ SBAR preparation

Expected Interventions

- ☐ Obtain ECG
- ☐ Administer diltiazem
- ☐ Begin infusion
- ☐ Notify provider

Expected Reassessment

- ☐ Rhythm
- ☐ Blood pressure
- ☐ Symptoms

Common Student Errors

- Misidentifying rhythm
- Delayed ECG
- Poor SBAR
- No reassessment

CJMM-ALIGNED SIMULATION DEBRIEFING QUICK REFERENCE GUIDE

This guide provides flexible reflection prompts aligned with the NCSBN Clinical Judgment Measurement Model (CJMM). Select, adapt, and sequence questions based on simulation objectives, student performance, and learner needs. Use the prompts as a cognitive aid to support discussion and reflection, not as a required script or checklist.

1. Recognize Cues
- What patient findings or changes in condition stood out to you?
- Which assessment findings were most important?
- What changes suggested that the patient's condition was improving or worsening?
- Were there any important cues that were initially missed or overlooked?

2. Analyze Cues
- What did the assessment findings suggest was happening with the patient?
- How were the important cues related to one another?
- How did the patient's pathophysiology help explain the findings?
- What additional information would have helped you better understand the patient's condition?

3. Prioritize Hypotheses
- What did you believe was the patient's highest-priority problem?
- Which patient problem or concern required attention first, and why?
- What was the greatest immediate risk to the patient?
- How did you decide which problem was most urgent?

4. Generate Solutions
- What nursing interventions did you consider for the patient's priority problem?
- What outcome did you expect from those interventions?
- What other interventions or approaches could have been considered?
- What additional resources, information, or team members could have helped guide the plan of care?

5. Take Action
- What action did you take first, and why?
- How did the patient's condition influence the order of your interventions?
- Were there any actions that should have been taken earlier or differently?
- How did communication and teamwork affect the care provided?

6. Evaluate Outcomes
- How did you determine whether the patient responded to the interventions?
- What findings needed to be reassessed after the interventions?
- Did the patient's response require a change in the plan of care?
- What would you continue to monitor or reassess if care of this patient continued?

7. Reflection and Future Clinical Application
- What went well, and what challenged you the most?
- What would you do differently if you encountered a similar situation again?
- How will you apply what you learned from this experience to future patient care?